Weight Loss, Weight Gain & Metabolic Health

A clinical teaching handbook for junior doctors and multidisciplinary healthcare professionals

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Scope: assessment, differential diagnosis, diagnostic testing, interpretation, nutrition, exercise, medical management, referral, and longitudinal monitoring. This handbook does not replace patient-specific evaluation, local clinical pathways, or specialty guidance.

How to use this handbook

Start with triage and diagnostic assessment. Determine whether weight change is intentional, whether a disease or medication is responsible, and whether the patient is at risk of malnutrition or an eating disorder. Only then select an intervention. Tests are indication-driven; avoid routine indiscriminate panels.

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1. Clinical foundations and physiology

Weight is not the same as adiposity

Weight comprises fat mass, lean tissue, water, bone and gut contents. A short-term weight increase may represent sodium-related fluid retention rather than fat gain. Weight loss can reflect fat loss, muscle loss, dehydration, or illness. Health goals should include function, cardiometabolic risk, muscle strength, nutritional adequacy and quality of life.

Energy regulation

Energy intake and expenditure interact with appetite regulation, sleep, medications, endocrine function, environment and adaptive thermogenesis. Total energy expenditure includes resting expenditure, physical activity, non-exercise activity and thermic effect of food. Calorie arithmetic is an initial planning approximation, not an exact prediction of body-weight change.

Definitions and terminology

- Overweight/obesity: use BMI as a screening measure, supplemented by waist circumference and clinical risk. BMI alone does not measure body composition.
- Unintentional weight loss: loss of >5% of usual body weight over 6–12 months is a common clinical trigger for investigation, especially in older adults; concerning symptoms require assessment sooner.
- Malnutrition: evaluate reduced intake/assimilation, weight loss, low muscle mass and inflammation/disease burden; consider validated GLIM criteria in appropriate settings.
- Sarcopenia: loss of muscle strength with reduced muscle quantity/quality, assessed using population-appropriate consensus methods.
- Weight cycling, stigma and disordered eating can worsen outcomes; use neutral, permission-based language.

2. Safety triage and history

Immediate or urgent escalation

- Rapid weight gain with dyspnea, orthopnea, edema or chest pain: evaluate possible fluid overload, heart failure or renal disease urgently.
- Unintentional weight loss with GI bleeding, persistent vomiting, dysphagia, severe dehydration, fever/night sweats, palpable mass or progressive weakness: prompt assessment.
- Hyperglycemia with vomiting, abdominal pain, deep breathing or altered consciousness: emergency evaluation for DKA/HHS.
- Severe food restriction, fainting, bradycardia, electrolyte disturbance, purging or suicidality: urgent eating-disorder/medical risk assessment.
- Severe malnutrition or prolonged minimal intake: assess refeeding syndrome risk before aggressive nutritional rehabilitation.

Core consultation framework

- Timeline: onset, magnitude, rate, intentionality, fluctuations, prior baseline, and prior interventions.
- Diet: 24-hour recall plus typical weekdays/weekends, portions, beverages, alcohol, access, cultural patterns, food insecurity.
- Appetite/GI: early satiety, nausea, vomiting, diarrhea, constipation, steatorrhea, dysphagia, abdominal pain.
- Activity and sleep: work patterns, step/activity trends, resistance training, shift work, snoring, daytime sleepiness.
- Medical: diabetes, thyroid, kidney/liver/heart disease, cancer symptoms, infections, reproductive/endocrine history.
- Medication review: prescription, OTC, supplements, steroids, insulin, antipsychotics, antidepressants, GLP-1 agents, stimulants.
- Psychosocial: mood, stress, body image, eating disorder symptoms, substance use, social support and preferences.
- Family history: diabetes, premature cardiovascular disease, obesity, inherited lipid disorders.

3. Measurement and anthropometry

Measure	How to obtain	Interpretation / caveat
Weight	Same calibrated scale, similar clothing/time; track trend	Day-to-day water changes common; use multiweek average
BMI	kg / m^2	Adults: <18.5 underweight; 18.5–24.9 conventional reference; 25–29.9 overweight; ≥30 obesity. Ethnicity-specific risk thresholds may be lower.
Waist	Standardized midpoint/iliac landmark protocol	Central adiposity and risk; use ethnicity- and guideline-specific thresholds

BP	Correct cuff, rested, repeat readings	Use guideline-specific hypertension classification and home readings
Body composition	DXA/BIA when useful and available	Hydration and device methods affect estimates
Function	Grip strength, chair rise, gait speed	Supports sarcopenia/frailty assessment
Weight change %	(current − baseline)/baseline × 100	Interpret timeframe, intentionality and fluid status

Clinical goals

For adults with excess adiposity, a sustained ~5% loss can improve several cardiometabolic risk markers; greater loss may be needed for some conditions. Goals must be individualized and should not assume that every patient benefits from weight loss. For underweight patients, prioritize restoring nutritional status and functional capacity rather than achieving a single BMI target.

4. Causes of weight gain

Cause / mechanism	Clues	Assessment / next step
Energy intake exceeding expenditure	Gradual gain; portions, liquid calories, reduced activity	Diet/activity history; collaborative plan
Sleep restriction / sleep apnea	Snoring, daytime sleepiness, short sleep	Sleep history; validated assessment/sleep study when indicated
Hypothyroidism	Cold intolerance, constipation, fatigue, dry skin	TSH, reflex free T4 if abnormal
Cushing syndrome	Proximal weakness, wide purple striae, easy bruising	Test only if discriminatory features; validated cortisol screening
PCOS	Irregular menses, hyperandrogenism	Clinical criteria and exclusion tests; metabolic risk assessment
Menopause/aging	Body composition shift, reduced activity	History; muscle and lifestyle assessment
Medication effect	Gain after starting insulin, glucocorticoids, some antipsychotics, etc.	Medication timeline; coordinate safer alternatives
Fluid retention	Rapid gain, edema, orthopnea, ascites	Exam; renal/liver/cardiac tests as indicated
Pregnancy	Amenorrhea or possibility of pregnancy	Pregnancy test when relevant
Depression/stress and binge eating	Mood changes, loss of control eating	Screen and refer for evidence-based care

Avoid common errors

Do not label all weight gain as endocrine disease. Do not use routine cortisol, insulin or sex-hormone panels without clinical indications. A normal TSH makes primary hypothyroidism less likely, but not all rare central causes.

5. Causes of unintentional weight loss

Etiology	Typical features	Targeted investigations
Inadequate intake / food insecurity	Reduced access, restrictive diet, dental issues	Nutrition/social assessment; oral/dental review
Malignancy	Progressive loss, bleeding, mass, night sweats	Age/risk-appropriate screening; symptom-directed imaging/endoscopy
Hyperthyroidism	Heat intolerance, tremor, tachycardia	TSH, free T4 ± T3
Diabetes with hyperglycemia	Polyuria, thirst, fatigue	Plasma glucose, HbA1c; ketones if acutely ill
Malabsorption / celiac	Chronic diarrhea, steatorrhea, anemia	CBC, ferritin, tTG-IgA + total IgA while eating gluten
Inflammatory bowel disease	Diarrhea, pain, blood in stool	CBC, CRP, fecal calprotectin; endoscopy

Chronic infection	Fever, exposure risks, cough	Exposure-guided HIV/TB and other testing
Depression / eating disorder	Mood, restrictive behaviors, distorted body image	Clinical interview and specialist assessment
Medication/substance effect	Temporal association, nausea, appetite suppression	Medication and substance review
Heart/lung/renal/liver disease	Dyspnea, edema, chronic symptoms	Exam and organ-specific tests
Adrenal insufficiency	Hypotension, nausea, hyperpigmentation	Morning cortisol/ACTH; confirmatory testing

Older adults

Assess cognition, swallowing, dentition, loneliness, polypharmacy, functional limitations and sarcopenia. Weight loss in older adults warrants attention even when BMI is not low.

6. Diagnostic investigations and interpretation

Choose investigations according to history and examination; no universal "weight panel" exists. Reference ranges vary by assay, lab, age, sex and pregnancy. Abnormal results need clinical confirmation and context.

Test	When to consider	Interpretation / pitfalls
CBC, indices	Fatigue, pallor, weight loss, suspected inflammation	Anemia patterns guide iron/B12/folate workup; WBC nonspecific
CMP: electrolytes, kidney, liver	Comorbidity, dehydration, edema, medication effects	Creatinine/eGFR, albumin, transaminases; assess trend and specimen quality
HbA1c / fasting glucose	Metabolic risk, polyuria, unexplained loss	Diabetes: HbA1c ≥6.5% or FPG ≥126 mg/dL; usually confirm without unequivocal hyperglycemia
Lipid profile	Cardiovascular risk assessment	LDL/non-HDL/triglycerides; interpret with global ASCVD risk
TSH ± free T4	Symptoms suggesting thyroid disease; selected evaluation	High TSH + low FT4: overt primary hypothyroidism; low TSH + high FT4/T3: thyrotoxicosis
Ferritin, iron studies	Anemia, fatigue, restrictive diet	Low ferritin supports iron deficiency; inflammation may elevate ferritin
B12 ± MMA	Neuropathy, macrocytosis, vegan diet, malabsorption	Borderline B12 may need MMA; renal impairment affects MMA
CRP/ESR	Suspected inflammatory condition	Nonspecific; not routine obesity biomarkers
Urine ACR	Diabetes, hypertension, kidney risk	≥30 mg/g is increased albuminuria; confirm persistence
Pregnancy test	Pregnancy possible and relevant to symptoms/treatment	Interpret timing and clinical context
Celiac serology	Diarrhea, iron deficiency, unexplained loss	tTG-IgA + total IgA; testing on gluten-containing diet
Cortisol testing	Specific features of Cushing or adrenal insufficiency	Use validated endocrine pathways; random cortisol unsuitable for Cushing screening
Sleep study	Suspected OSA	AHI with symptoms/comorbidity; specialist interpretation
Stool tests/endoscopy/imaging	Alarm symptoms or targeted concern	Not screening tests for every weight-change presentation

Clinical interpretation workflow

- Verify identity, date, specimen, fasting status, units, method and laboratory reference range.
- Check for urgency: critical potassium, severe sodium disturbance, marked hyperglycemia with illness, severe anemia or other critical findings require appropriate escalation.

- Separate screening, diagnostic and monitoring uses; do not treat abnormal screening results as definitive diagnoses.
- Assess pretest probability and confounders (pregnancy, medications, acute illness, hemolysis, kidney disease).
- Compare longitudinal trends, repeat/confirm when appropriate, and document uncertainty.

7. Energy balance and nutrition prescription

Initial energy estimation

Mifflin–St Jeor resting energy expenditure (kcal/day): men = 10×kg + 6.25×cm − 5×age + 5; women = 10×kg + 6.25×cm − 5×age − 161. This is an estimate, not a diagnostic measurement; it may be inaccurate in unusual body composition, pregnancy, acute illness or major fluid shifts. Estimate total energy expenditure with activity context and adjust to observed 2–4 week trends.

Fat-loss nutrition

A modest individualized energy deficit (often ~300–500 kcal/day as an initial planning range) may support sustainable loss; larger deficits or very-low-calorie diets require clinical oversight. Avoid prescriptive deficits in patients with eating disorders, pregnancy, active malnutrition or other contraindications. Prioritize adequate protein, fiber-rich foods, fruit/vegetables, minimally processed staples and sustainable preferences.

Weight-gain nutrition

For patients with inadequate intake or low body weight, a cautious surplus (often ~250–500 kcal/day when clinically appropriate) can be a starting estimate, adjusted for tolerance, goals and response. Do not use aggressive refeeding in high-risk malnutrition. Offer frequent meals/snacks, energy-dense nutritious foods, and oral nutrition supplements if indicated.

Protein, carbohydrate, fat and fiber

- General adult protein RDA is ~0.8 g/kg/day; resistance-training goals often use ~1.2–1.6 g/kg/day or higher in selected healthy adults, individualized for renal disease and other clinical conditions.
- Distribute protein across meals and pair with progressive resistance exercise for muscle gain/preservation.
- Carbohydrates can support activity and dietary adherence; choose high-fiber sources and individualize for diabetes medications.
- Include unsaturated fats; reduce trans fats and excess saturated fat based on cardiometabolic risk.
- A common adult fiber target is ~14 g per 1,000 kcal, advanced gradually with adequate fluid as tolerated.
- Avoid unnecessary supplement stacks, detoxes, diuretics, laxatives or extreme restriction.

Diet patterns

Mediterranean-style, DASH, culturally adapted balanced diets and other nutritionally adequate patterns can work. The most effective plan is safe, sustainable and compatible with patient values, budget and health needs.

8. Evidence-based fat-loss care

Stepwise clinical pathway

- 1. Confirm indication: adiposity-related health risk, patient preference, contraindications, and alternative priorities.
- 2. Establish baseline: weight trend, waist, BP, metabolic risk, medications, nutrition, sleep, physical activity and mental health.
- 3. Agree on functional and health goals; use shared decision-making and avoid stigmatizing language.
- 4. Implement dietary change, resistance exercise, aerobic activity, sleep support and behavior skills.
- 5. Review response and tolerability at ~2–4 weeks initially; adjust based on multiweek trends.
- 6. Consider specialist referral and anti-obesity medications when guideline eligibility and patient circumstances support them.
- 7. Plan maintenance early: ongoing support, relapse prevention and management of chronic obesity where applicable.

Behavior change methods

Use self-monitoring (optional if not triggering), implementation intentions, environmental changes, problem solving, meal planning, motivational interviewing, and realistic habit goals. Consider behavioral therapy for binge eating or other eating disorders.

Rate of change

For many adults, ~0.25–1% of body weight per week may be a practical initial range, but this is not a universal target. Slower loss may be preferable, and rapid changes may reflect fluid shifts or illness.

9. Healthy weight gain and muscle development

First establish the reason for low weight

Differentiate constitutional thinness, inadequate intake, high expenditure, malabsorption, endocrine disease, chronic illness and eating disorders. Do not prescribe a surplus before assessing alarm symptoms or refeeding risk.

Practical intervention

- Aim for adequate total energy and protein using meals/snacks that the patient can tolerate.
- Use nutrient-dense additions: dairy/fortified alternatives, nuts, seeds, legumes, olive oil, eggs, fish or other preferred protein sources.
- Progressive resistance training 2–3+ days/week when safe, with gradual load progression and recovery.
- Monitor strength, function, intake, GI symptoms and body-weight trend, not weight alone.
- Refer to a registered dietitian for complex malnutrition, chronic disease or persistent inadequate intake.

Refeeding syndrome warning

Patients with very low BMI, major recent weight loss, negligible intake or electrolyte depletion may be at risk of refeeding syndrome. Assess risk using local/NICE/ASPEN protocols; check phosphate, potassium, magnesium and fluid status; use clinician-supervised initiation and monitoring, with thiamine where indicated.

10. Exercise and activity prescription

Component	Typical adult target	Clinical adaptations
Aerobic	WHO: 150–300 min/week moderate or 75–150 vigorous, as tolerated	Start below target for sedentary or frail patients; progress gradually
Strength	Major muscle groups ≥2 days/week	Prioritize technique and progressive overload; adapt to joint disease
Sedentary time	Break up prolonged sitting	Short walking/activity breaks; mobility limitations considered
Balance	Especially important in older adults	Balance and functional training ~3 days/week when indicated
Recovery	Adequate sleep, rest and nutrition	Monitor injury, fatigue, pain and overtraining

Exercise safety

Cardiopulmonary symptoms, unstable chronic disease, recent acute events and severe malnutrition warrant individualized clearance or supervised rehabilitation. Exercise is beneficial independent of weight loss; do not use exercise as punishment for food intake.

11. Special populations

Population	Priority considerations
Pregnancy / postpartum	Avoid routine weight-loss diets during pregnancy; use gestational weight guidance and obstetric supervision; consider lactation needs postpartum.
Children / adolescents	Use age/sex growth curves and pediatric protocols, family-centered care, avoid adult BMI cutoffs and restrictive plans.
Older adults	Prevent sarcopenia, falls and malnutrition; prioritize protein, strength and function; avoid unnecessary aggressive loss.
Type 2 diabetes	Adjust nutrition with glucose-lowering medication to prevent hypoglycemia; coordinate changes in insulin/sulfonylureas.
CKD	Individualize protein, potassium, sodium and fluid with nephrology/dietetics; no universal high-protein plan.
Heart failure / edema	Weight shifts may reflect fluid; use clinical congestion assessment and individualized sodium/fluid advice.
Eating disorders	Specialist-led treatment; avoid calorie targets and weight tracking that may reinforce pathology.
Post-bariatric surgery	Lifelong nutrition monitoring, protein adequacy, micronutrients and procedure-specific supplementation.

12. Pharmacotherapy and bariatric care

Anti-obesity medications

When obesity-related risk and guideline criteria support use, discuss evidence-based medications (including incretin-based therapies where locally approved), benefits, adverse effects, contraindications, cost, access and long-term follow-up. Eligibility thresholds differ by country, ethnicity, comorbidity and product label. These medications require clinician prescribing and monitoring; do not let an agent recommend a prescription autonomously.

Medication monitoring

- Assess baseline contraindications, pregnancy plans, medication interactions and gastrointestinal history.
- Monitor response, tolerability, hydration, nutrition and potential lean-mass loss.
- Reassess when inadequate response or adverse effects occur; follow the specific product label and current guideline.
- Avoid compounded/unapproved products without robust safety and regulatory review.

Metabolic/bariatric surgery

Specialist multidisciplinary evaluation is appropriate for selected patients with obesity and/or metabolic disease. Eligibility and procedural choice depend on current guidelines, BMI, ethnicity, comorbidity, prior therapy and patient preference. Discuss nutritional deficiency risk, surgical complications and lifelong follow-up.

13. Cardiometabolic health and prevention

Beyond the scale

Assess BP, lipids, glucose status, sleep apnea, fatty liver risk, tobacco/alcohol, fitness, function, psychosocial health and preventive screening. Favor interventions that improve health even when weight is stable.

Clinical risk interpretation

- Use validated cardiovascular risk estimation appropriate to jurisdiction and population; avoid interpreting LDL alone without risk context.
- MASLD risk: consider fibrosis risk stratification (e.g., FIB-4 in appropriate adults) when metabolic risk and liver disease warrant assessment; recognize age and acute-illness limitations.

- For diabetes, monitor kidney health (eGFR, urine ACR), eye and foot health per guidelines.
- Screen for obstructive sleep apnea when symptoms and risk factors justify it.

14. Follow-up, plateaus and relapse

Suggested review schedule

At 2–4 weeks: adherence, appetite, sleep, adverse effects, activity, weight trend and symptoms. At 8–12 weeks: reassess functional goals, waist/BP and relevant biomarkers. Longer term: maintenance, chronic disease care and periodic medication/nutrition review. Timing varies by illness severity and intervention.

When progress stalls

- Check measurement consistency and distinguish fluid fluctuation from true plateau.
- Review hunger, sleep, stress, medication changes, adherence, activity compensation and energy needs.
- Assess whether new symptoms warrant disease evaluation.
- Adjust the plan collaboratively; do not automatically escalate restriction.
- For weight gain, reassess intake adequacy, GI tolerance, resistance training and missed medical causes.

15. Worked clinical cases

Case A: Gradual weight gain

Presentation: 34-year-old with 8 kg gain over 18 months, sedentary work, 5-hour sleep, no edema or discriminatory endocrine signs.

Clinical reasoning: Confirm weight trend, medications, diet, sleep, BP, waist and cardiometabolic risk. Consider HbA1c/lipids based on risk. Do not order routine cortisol testing.

Next step: Collaborative nutrition/activity/sleep plan; review after 4 weeks.

Case B: Rapid weight gain with breathlessness

Presentation: 67-year-old with 4 kg increase in 6 days, ankle edema and orthopnea.

Clinical reasoning: Treat as possible fluid retention/heart failure; prompt clinical assessment with vitals, exam, renal/electrolyte tests, ECG and cardiac evaluation as indicated.

Next step: Do not prescribe a calorie deficit as the initial response.

Case C: Unintentional loss with polyuria

Presentation: 46-year-old with 7% weight loss over 3 months, thirst and frequent urination.

Clinical reasoning: Check glucose urgently; assess ketones/acid-base status if acutely unwell. HbA1c and confirmatory diabetes evaluation as appropriate.

Next step: Escalate immediately if vomiting, dehydration, altered consciousness or DKA symptoms.

Case D: Low BMI and restricted intake

Presentation: 23-year-old with BMI 16.5, recent 12% loss, dizziness and minimal intake.

Clinical reasoning: Assess eating disorder, vitals, ECG, electrolytes incl. phosphate/magnesium, hydration and refeeding risk.

Next step: Urgent supervised nutritional/medical care if unstable; no unsupervised large calorie surplus.

Case E: Weight gain with thyroid symptoms

Presentation: 40-year-old with constipation, cold intolerance, fatigue and gradual gain.

Clinical reasoning: TSH and free T4; elevated TSH with low FT4 supports primary hypothyroidism.

Next step: Treat thyroid disease per clinician guidance; weight change alone is not a diagnostic test.

Case F: Strength-focused weight gain

Presentation: 28-year-old healthy adult with stable low-normal BMI, no alarm symptoms, seeks muscle gain.

Clinical reasoning: Assess diet, training history, baseline strength and relevant clinical risks; avoid unnecessary laboratory panels.

Next step: Small energy surplus, adequate protein, progressive resistance training and monthly monitoring.

16. Patient education and clinical documentation

Teach-back counseling

Ask: “What is the main goal you want to work on?” “What changes feel realistic this month?” “What would make this difficult?” Confirm understanding of warning signs, follow-up and when to seek care. Use culturally appropriate foods and accessible exercise options.

Structured visit note

- Reason for visit and patient-defined goals
- Weight timeline, intent, symptoms and red flags
- Relevant medical, medication, psychosocial and dietary history
- Exam and measurements (with method/date)
- Problem representation and prioritized differential
- Tests ordered with indication and interpretation plan
- Shared care plan, referral, safety-net instructions and follow-up date

17. Medical agent implementation and validation

Recommended architecture

Separate structured rules from narrative retrieval. Intake agent captures history and validates units; deterministic triage checks emergencies; guideline retrieval supplies source-backed clinical pathways; interpretation service handles lab values with method-specific thresholds; clinician-facing summarizer presents uncertainty and alternatives.

Required output fields

- Presenting concern, weight-change magnitude and timeline, intentionality
- Red flags and escalation status (never overridden by LLM)
- Supporting and contradictory evidence for each differential
- Recommended investigations with indication and contraindications
- Lab interpretation with units, assay and reference source
- Proposed next action, follow-up and referral urgency
- Source guideline, version, jurisdiction and last clinical review date
- Explicit uncertainty, missing data and clinician approval status

Safety constraints

- Never diagnose from BMI, a single symptom or a single nonspecific biomarker alone.
- Never generate a medication prescription, extreme calorie target or refeeding plan without clinician oversight.
- Do not infer age, pregnancy, kidney function or medications when missing.
- Prevent hallucinated references and stale guideline thresholds through versioned retrieval.
- Protect health information, restrict access and audit clinical decisions.
- Evaluate against clinician-adjudicated cases for missed emergencies, inappropriate tests, unsafe advice and subgroup performance.

18. References and update policy

The following are primary starting points for a formal reference library. This educational edition was not subjected to a systematic current-guideline review; verify the latest revision and regional applicability before clinical use.

WHO — Healthy diet and physical activity recommendations. https://www.who.int/news-room/fact-sheets/detail/healthy-diet

WHO — Guidelines on physical activity and sedentary behaviour. https://www.who.int/publications/i/item/9789240015128

NICE — Overweight and obesity management guidance. https://www.nice.org.uk/guidance

NICE — Nutrition support in adults (CG32). https://www.nice.org.uk/guidance/cg32

ADA — Standards of Care in Diabetes. https://professional.diabetes.org/standards-of-care

KDIGO — CKD clinical practice guidelines. https://kdigo.org/guidelines/

AACE — Endocrine and obesity clinical guidance. https://pro.aace.com/clinical-guidance

ASPEN — Nutrition support and refeeding resources. https://nutritioncare.org/

ESPEN — Clinical nutrition guidelines. https://www.espen.org/guidelines-home/espen-guidelines

ACSM — Exercise prescription and professional resources. https://www.acsm.org/

NIH NIDDK — Weight management and digestive/endocrine conditions. https://www.niddk.nih.gov/

GLIM — Global Leadership Initiative on Malnutrition. https://www.glimcriteria.org/

Clinical editorial checklist

- Clinical specialist review of each diagnostic pathway
- Verification of thresholds, units, population applicability and references
- Jurisdiction-specific medication labels and eligibility
- Safety review for pregnancy, adolescents, eating disorders and malnutrition
- Scheduled updates and post-deployment error reporting

End of handbook.